[redacted]

[redacted]

TCGA-18-3416

[redacted]

Surgical Pathology Consultation Report

* Addended *

SPECIMEN(S) RECEIVED
1. Lymph-Node: ST9 Inf Pulm Ligament/[redacted]
2. Lymph node: LT TB ANGLE ST10L
3. Lymph node: sump node/QS
   LUNG: Lt upper lobe broncial margin[redacted]
4. Lymph node: lymph node neck

DIAGNOSIS
1. - 3. ST9 inferior pulmonary ligament, LT TB angle ST10L, sump node, lymph node dissection:
- Negative for malignancy, lymph node. (x3)

4. Left upper lobe lung lobectomy:
- Squamous cell carcinoma, poorly differentiated, 8 cm in greatest diameter. (pT2)
- Bronchial resection margin negative for tumor.
- All 5 lobar lymph nodes negative for tumor.
- 2 of 6 segmental lymph node involved by tumor. (pN1)
- 1 subsegmental lymph node negative for tumor.
- Many lymph nodes show marked anthracosis with nodular fibrosis.
- Remaining lung parenchyma shows anthracotic deposits in wall of terminal/respiratory bronchioles.

5. Lymph node neck biopsy:
- Normal thyroid tissue, negative for malignancy.

SYNOPTIC DATA

Specimen Type:	Lobectomy
Laterality:	Left
Tumor Site:	LUL

[redacted]

[redacted]

[redacted]

Tumor Size: Greatest dimension: 8.0 cm
Histologic Type: Squamous cell carcinoma
Histologic Grade: G3: Poorly differentiated
Pathologic Staging (pTNM): pT2: Tumor with any of the following features of size or extent: greater than 3 cm in greatest dimension; involves main bronchus, 2 cm or more distal to the carina; invades the visceral pleura; associated with atelectasis or obstructive pneumonitis that extends to the hilar region but does not involve the entire lung
pN1: Metastasis in ipsilateral peribronchial and/or ipsilateral hilar lymph nodes, including intrapulmonary nodes involved by direct extension of the primary tumor
Lymph node stations Involved: 13L (Left segmental)
pMX: Cannot be assessed
Margins: Margins uninvolved by invasive carcinoma
Direct Extension of Tumor: None identified
Blood Vessel Invasion: Absent
Lymphatic (Small Vessel) Invasion (L): Absent
Additional Pathologic Findings: Other: Marked anthracosis of peribronchial lymph nodes
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COMMENT
The segmental lymph nodes involvement could be by direct invasion.

[redacted]

CLINICAL HISTORY
[redacted]

GROSS DESCRIPTION
1. The specimen labeled with the patient's name and as "Lymph-Node: ST9 Inf Pulm Ligament/qs". Consists of one lymph node covered by few fragments of yellow-tan adipose tissue, measuring 1.5 x 1.0 x 0.7 cm; ceived in 10% buffered formalin. Cross sections of the lymph node are nomogeneously black and firm.

[redacted]

[redacted]

[redacted]

[redacted] specimen is bisected and submitted in toto

2. The specimen labeled with the patient's name and as "Lymph node: LT TB ANGLE ST10L". Consists of one lymph node covered by few fragments of yellow-tan adipose tissue, measuring 1.8 x 1.1 x 0.3 cm; received in 10% buffered formalin. Cross sections of the lymph node are homogeneously black and firm.
2A specimen is bisected and submitted in toto

3. The specimen labeled with the patient's name and as "Lymph node: sump node/QS". Consists of two pieces of oval, gray-black, soft tissue fragments each measuring up to 1.0 cm; received fresh. This is examined for intraoperative consultation by frozen section, on the entire specimen in one block.
3A frozen section block, resubmitted toto

4. The specimen labeled with the patient's name and as "LUNG: Lt upper lobe bronchial margin, [redacted] The specimen is received fresh and subsequently placed in to 10% buffered formalin. It consists of a left upper lobe of lung, measuring 16.5 x 15.5 x 4.0 cm. The pleural surface is focally hemorrhagic and ranges in color from pink-tan to purple, showing diffuse areas of black pigmentation. Some fibrous adhesions are also noted. An area of infarct/ ? strip of parietal pleura is noted, measuring 4.3 cm in length x 1.5 cm in diameter. The surface of this area is homogeneously white, smooth and firm. Cross-sections reveal a well-circumscribed tumor, measuring 8.0 x 3.8 x 3.3 cm. Cross sections of the tumor are homogeneously white-tan and firm. Immediately adjacent to the tumor a small yellow, firm area is noted. The tumor is located approximately 1.5 cm from the bronchial resection margin, 1.0 cm from the closest pleural surface and 0.7 cm from the pulmonary artery resection margin. Further sectioning reveals multiple small-indurated areas, which are better palpated than seen. The bronchi all appear slightly and dilated and partially calcified. Cross sections of the remaining lung parenchyma are homogeneously brown-tan and grossly unremarkable. Multiple probable lymph nodes are identified, ranging from 0.3 to 2.0 cm. The bronchial resection margin has been taken en face and submitted for intraoperative consultation by frozen section. Two large pieces of lung tissue has also been taken for an asbestos fiber count (requested by [redacted] Representative sections are submitted as follows:
4A frozen section block, resubmitted
4B pulmonary artery resection margin, en face

[redacted]

[redacted]

[redacted]

[redacted]

4C representative sections of pleural adhesions
4D-4E tumor with ? parietal pleura/area of infarct
4F tumor with closest pleural surface
4G-4H tumor with adjacent normal parenchyma
4I representative sections of fibrous/indurated areas
4J-4K representative sections of normal lung parenchyma
4L one representative section from base of lung
4M representative sections of various indurated areas
4N-4P one bisected probable lymph node are block, lobar region
4Q-4R one trisected probable lymph node, lobar region
4S-4T two probable lymph nodes per block, segmental region
'-4V one trisected probable lymph node per block, segmental region
.W-4X two probable lymph nodes per block, subsegmental region

5. The specimen labeled with the patient's name and as "Lymph node: lymph node neck". Consists of one unoriented, irregularly shaped piece of yellow-tan adipose tissue, measuring 1.4 x 0.8 x 0.5 cm; received in 10% buffered formalin. Cross sections reveal one probable lymph node, measuring 0.9 x 0.5 x 0.5 cm. The cut surface of the lymph node is homogeneously tan and firm.
5A specimen is bisected and submitted in toto

[redacted]

QUICK SECTION
QS3A: "sump" node: Lymph nodes, negative for malignancy
QS4A: LUL: bronchial resection margin: negative for malignancy.

[redacted]

Addendum Comment
A large number of ferruginous (asbestos) bodies are found in practically all sections with lung parenchyma tissue.

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[redacted]

[redacted]